Brigatinib monotherapy · ALK-positive advanced non-small-cell lung cancer (NSCLC) Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00562 · VERSION 4

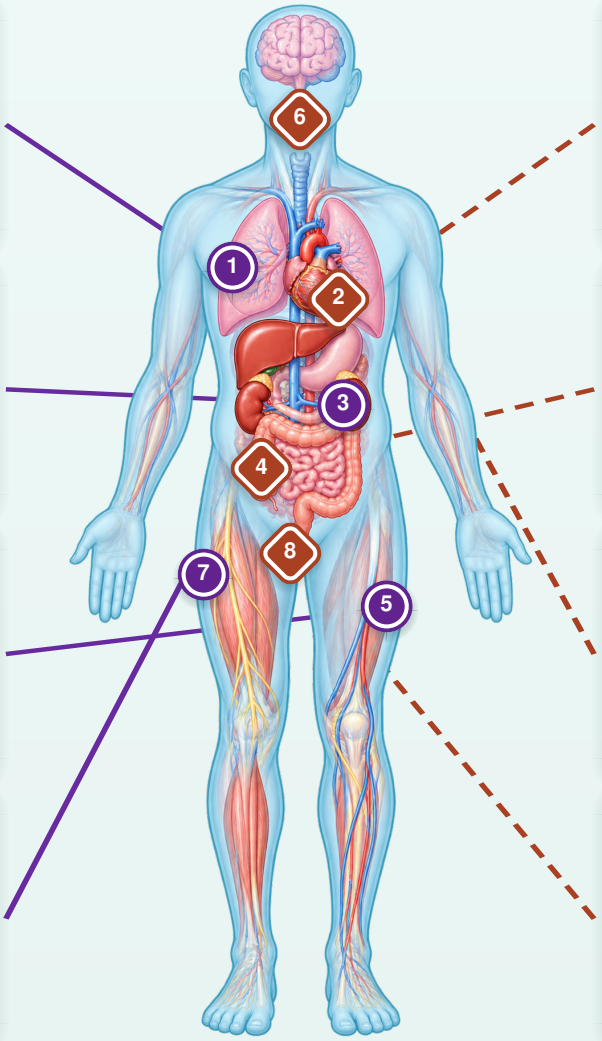
Brigatinib Therapy

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Oral ALK-targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Breathing

Brigatinib

New cough, breathlessness or fever needs immediate advice, especially during the first week.

2 Heart / blood pressure

Brigatinib

Report fainting, slow heartbeat, chest pain or severe headache.

3 Liver tests

Brigatinib

Blood tests check the liver; report jaundice, dark urine or marked illness.

4 Glucose / pancreatic enzymes

Brigatinib

Report unusual thirst, frequent urination or severe abdominal pain.

5 Muscles

Brigatinib

Report unexplained muscle pain or weakness.

6 Vision

Brigatinib

Report new or worsening blurred vision or other changes.

7 Skin / sun sensitivity

Brigatinib

Use sun protection and report severe rash or sun reaction.

8 Bowel / nausea

Brigatinib

Report persistent vomiting or severe diarrhoea; keep fluids up as advised.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.

Checks and follow-up

Before starting: ALK result, blood count, kidney/liver tests, breathing-function assessment, electrocardiogram (ECG), blood pressure, fasting glucose, muscle enzyme creatine phosphokinase (CPK) and pancreatic enzymes. Review breathing symptoms on days 8 and 15, then monthly or sooner if needed. ECG and blood pressure are checked every two weeks initially, then monthly. Liver tests are every two weeks for the first three months, then as advised. Blood count, kidneys, fasting glucose, CPK and pancreatic enzymes are usually checked monthly. Scans follow your Consultant's plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

One written tablet plan

Keep the current dose, tablet strengths and next review clear. Ask the pharmacist about safe storage, handling and returning unused tablets. Do not share medicines.

Anti-sickness medicines only if needed

Brigatinib has minimal-to-low oral sickness risk. The oral NCCP guidance uses anti-sickness medicines as required; your team chooses a suitable medicine, dose and duration. Persistent vomiting still needs advice.

Food and other medicines

Take with or without food; avoid grapefruit. Check every prescription, herbal or non-prescription product, including St John's wort, before starting or stopping it.

Skin and sunshine

Avoid prolonged sun exposure during treatment and for at least five days afterwards. Use protective clothing, a hat, broad-spectrum sun protection factor (SPF) 30 or higher sunscreen and lip balm. Report severe rash or sun reaction.

Symptoms and practical support

Keep the 24-hour oncology contact available. Ask early for help with symptoms, eating, tablet supply, work, travel or emotional wellbeing.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00562/

Public information only. No personal information is encoded.